



SURGICAL PATHOLOGY REPORT

Patient Name:

Med. Rec. #:

DOB: (Age:

Gender: M

Ref. Physician:

Patient Address:

Ref. Source:

Accession #:

Date of Procedure:

Date of Receipt:

Date of Report:

Account #:

Billing Type:

Additional Copy to:

Clinical Diagnosis & History:

year old male with cT1C Gleason 3+3=6, PSA 4.02 and T2 bladder carcinoma for partial cystectomy and open radical retropubic prostatectomy, bilateral lymph node dissection.

Specimens Submitted:

- 1: SP: Dome of bladder, urachus; partial cystectomy (fs)
- 2: SP: Right pelvic lymph nodes; dissection
- 3: SP: Right common iliac lymph nodes; dissection
- 4: SP: Left pelvic lymph nodes; dissection
- 5: SP: Bladder margin; biopsy (fs)
- 6: SP: Bladder and prostate and seminal vesicles; prostatectomy and completion cystectomy
- 7: SP: Right vas deferens; excision
- 8: SP: Left vas deferens; excision
- 9: SP: Soft tissue, cord; excision

DIAGNOSIS:

1. SP: Dome of bladder, urachus; partial cystectomy (fs):

Tumor Type:

Invasive urothelial carcinoma, NOS

Histologic Grade:

High grade

Pattern of growth of the Non-Invasive component:

In situ component not identified

Pattern of growth of the Invasive component:

Infiltrating

Tumor Multicentricity:

Not identified

Bladder Local Invasion:

Perivesical soft tissues

Extravesical Tumor Extension:

See Part 6

Vascular Invasion:

Not identified

Perineural Invasion:

Not identified

Surgical Margins:

Right anterior bladder margin positive, Right and left posterior bladder margins negative.

*ICD-3
Carcinoma, urothelial NOS 8120/3
Site ⁶⁷¹ Bladder dome C67.1
~~path~~ Bladder NOS C67.9
JW 4/20/14*

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Non-Neoplastic Mucosa:

Exhibiting ulceration
Extensively denuded urothelium

Prostate:

Other See Prostate template

Seminal Vesicles:

Other See prostate template

Perivesical Lymph Nodes:

LN Not involved see comment

The Pathologic Stage is (AJCC 2002):

pT3 (Invasion of perivesicle soft tissue)

PR0 (No involvement of prostate)

Comment: Rare cytokeratin positive cells are noted in one lymph node; it is difficult to ascertain the exact nature of these cells

Some of the immunohistochemistry and ISH tests were developed and their performance characteristics were determined by the Department of Pathology. They have not been cleared or approved by the US Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or for research. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA '88) as qualified to perform high complexity clinical laboratory testing.

2. SP: Right pelvic lymph nodes; dissection:

Lymph Node Dissection:

Benign lymph nodes

Number of lymph nodes examined: 10

3. SP: Right common iliac lymph nodes; dissection:

Lymph Node Dissection:

Benign fibroadipose tissue

The specimen is entirely submitted.

4. SP: Left pelvic lymph nodes; dissection:

Lymph Node Dissection:

Benign lymph nodes

Metastatic Urothelial carcinoma

Number of lymph nodes examined: 8

Number of lymph nodes with metastatic disease: 1

Size of the largest lymph node involved by tumor: 0.9cm.

Size of the largest metastatic focus : 0.8 cm.

Extranodal extension is not identified

5. SP: Bladder, margin; Partial resection (fs):

-Prominently denuded bladder urothelium with reactive atypia

-No carcinoma seen

6. SP: Bladder and prostate and seminal vesicles; prostatectomy and completion cystectomy:

Tumor Type:

Adenocarcinoma

Gleason's Grade:

Primary Gleason grade:3

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Secondary Gleason grade:3
Total Gleason score:6

Tumor Location:

Involves Right anterior

Involves Left anterior

Dominant tumor mass located in: Right and left anterior, apex to mid

Vascular Invasion:

Not Identified

Perineural Invasion:

Identified

Tumor Multicentricity:

Multicentric foci of invasive carcinoma are present

High Grade Prostatic Intraepithelial Neoplasia:

Identified

Capsule:

Tumor invades into, but not beyond prostate capsule

Seminal Vesicles:

Not involved

Bladder Neck:

Not Involved

Surgical Margins:

Free of tumor

Non-Neoplastic Prostate:

Exhibits nodular hyperplasia

Necrotizing granulomas in prostate consistent with history of prior BCG therapy, AFB stain is negative for acid fast bacilli

Staging (AJCC 1997):

pT2b (confined to the prostate & capsule, involving both lobes)

Portion of urinary bladder with reactive urothelial atypia

- Benign ureters

No carcinoma is present

7. SP: Right vas deferens; Excision:

-Benign portion of vas deferens

8. SP: Left vas deferens; Excision:

-Benign portion of vas deferens

9. SP: Soft tissue, cord; Excision:

-Mature adipose tissue, consistent with lipoma

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

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Gross Description:

1.) The specimen is received fresh for frozen consultation, labeled "Dome of bladder and urachus, black stitch marks posterior wall and white stitch marks right wall". It consists of an irregular portion of the dome of bladder, urachus and surrounding fibrofatty tissue measuring 18.0 cm from superior to inferior, 7.8 cm laterally and 5.9 cm from anterior to posterior. The bladder measures 6.0 x 5.5 x 2.6 cm. The bladder resection margins are inked as follows: right posterior – black, right lateral –green, right anterior-red, left anterior - black, left posterior-orange. Representative resection margins (approximately 50%) are shaved and submitted for frozen diagnosis. The residual bladder resection margins are submitted for routine processing. The bladder is opened along the median umbilical ligament to reveal a 5.6 x 5.2 x 2.6 cm white firm mass which grossly involves the entire bladder. The tumor invades perivesicular fat but does not reach the soft tissue resection margin grossly. No ureters and orifices are identified. Only a rim (0.3cm) of smooth bladder mucosa is present grossly. The specimen is photographed. TPS is submitted.

Summary of sections:

FSCA- left posterior margin
FSCB- right posterior margin
FSCC- right anterior margin
RLM – residual right lateral margin
RPM – residual right posterior margin
LPM – residual left posterior margin
LAM – residual left anterior margin
T - tumor
RM- rim of smooth mucosa
LN – possible lymph nodes

2). The specimen is received fresh, labeled "right pelvic lymph node (fresh and sterile)" and consists of multiple pink tan firm lymph nodes ranging from 0.9 to 3.5 cm in greatest dimension. All identified lymph nodes are submitted in nine cassettes.

Summary of sections:

LN - lymph nodes
QLN - quadrisected lymph node

3). The specimen is received fresh, labeled "right common iliac lymph nodes, fresh and sterile" and consists of a 4 x 4 x 1 cm aggregate of yellow-tan to red-pink, lobulated fibroadipose tissue with no lymph nodes identified. The specimen is entirely submitted in three cassettes.

Summary of sections:

U – undesignated

4). The specimen is received fresh, labeled "left pelvic lymph nodes, fresh and sterile" and consists of six pink tan firm lymph nodes ranging from 1.5 to 4.3 cm in greatest dimension. All identified lymph nodes are submitted in eight cassettes.

Summary of sections:

LN - lymph nodes
QLN - quadrisected lymph node

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5). The specimen is received fresh for frozen section consultation, labeled "Bladder margin" and consists of a 4.2 x 1.8 x 0.9cm portion of pink white bladder mucosa and soft tissue. The margin is submitted for frozen section consultation, the remaining tissue is entirely submitted for routine processing.

Summary of sections:

FSC - frozen section control

U - undesignated

6). The specimen is received fresh, labeled "Bladder and Prostate, seminal vesicles".

It consists of a partial cystectomy with attached prostate and seminal vesicles.

The total weight of the prostate is 47.5 grams.

The prostate measures 4 cm from apex to base, 4.5 cm transversely, and 4 cm from anterior to posterior.

The external surface of the prostate is smooth.

The prostate is inked (right=green, left=blue) and fixed in formalin overnight.

The prostate is serially cut from apex to base at approximately 3-4 mm intervals.

The prostate is entirely submitted.

Summary of Sections:

RA-right apical margin

LA-left apical margin

BN-bladder neck margin

RSV-right seminal vesicle

LSV-left seminal vesicle

A- -serial sections of prostate (apex to base)

The completion cystectomy measures 6 x 1.5 cm. There is a line of sutures measuring 6 cm in length. The bladder mucosa is red and no gross lesions are identified. Possible ureters are identified on the right and left side of the bladder, measuring 0.2 cm in length and 0.2 cm in diameter. On the left side of the bladder is a tubular structure measuring 2.0 cm in length, likely representing the vas deferens. The bladder mucosa is pink and edematous. Representative sections are submitted of the bladder.

RB – right bladder

LB – left bladder

V – probable left vas deferens

RU – right ureter margin, en face

RUO – right ureteral orifice

LU – left ureter

LUO – left ureteral orifice

7.) The specimen is received in formalin, labeled "Right vas deferens" and consists of a 7.4 cm in length and 0.3 cm in diameter of rubbery, tan-white cylindrical tissue, partially surrounded by adipose tissue, consistent with vas deferens. No lesions are identified grossly. A representative section is submitted.

Summary of sections:

RVD – right vas deferens

8.) The specimen is received in formalin, labeled "Left vas deferens" and consists of a 6.0 cm in length and 0.3 cm in diameter of rubbery, tan-white cylindrical tissue, partially surrounded by adipose tissue, consistent with vas deferens. No lesions are identified grossly. A representative section is submitted.

Summary of sections:

LVD -- left vas deferens

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9.) The specimen is received in formalin, labeled "Lipoma of the cord" and consists of a 15.4 x 5.8 x 1.5 cm fragment of focally hemorrhagic adipose tissue, partially surrounded by a thin translucent membranous which is sectioned to reveal no grossly identifiable lesions. A representative section is submitted.

Summary of sections:

L – lipoma

Summary of Sections:

Part 1: SP: Dome of bladder, urachus; partial cystectomy (fs)

Blocks	Block Designation	PCs
1	FSCA	1
1	FSCB	1
1	FSCC	1
1	LAM	1
3	LN	3
1	LPM	1
1	RLM	1
1	RM	1
1	RPM	1
5	T	5

Part 2: SP: Right pelvic lymph nodes; dissection

Blocks	Block Designation	PCs
5	LN	8
4	QLN	4

Part 3: SP: Right common iliac lymph nodes; dissection

Blocks	Block Designation	PCs
3	U	30

Part 4: SP: Left pelvic lymph nodes; dissection

Blocks	Block Designation	PCs
4	LN	5
4	QLN	4

Part 5: SP: Bladder margin; biopsy (fs)

Blocks	Block Designation	PCs
1	fsc	2
5	u	5

Part 6: SP: Bladder and prostate and seminal vesicles; prostatectomy and completion cystectomy

Blocks	Block Designation	PCs
1	A	1
1	B	1
1	C	1
1	D	1
1	E	1
1	F	1
1	G	1
1	LA	1
2	LB	2
2	LU	2
2	LUO	2

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1	RA	1
2	RB	2
1	RU	1
3	RUO	3
1	V	1

Part 7: SP: Right vas deferens; excision

Blocks	Block Designation	PCs
1	RVD	1

Part 8: SP: Left vas deferens; excision

Blocks	Block Designation	PCs
1	LVD	1

Part 9: SP: Soft tissue, cord; excision

Blocks	Block Designation	PCs
1	L	1

Special Studies:

Result	Special Stain	Comment
	AE1:AE3	
	NEG CONT	
	IMM RECUT	
	AFB CO+/-	

Intraoperative Consultation:

1. SP: DOME OF BLADDER, URACHUS (FS): RIGHT ANTERIOR MARGIN – SMALL FOCUS OF ATYPICAL EPITHELIAL CELLS IN LAMINA PROPRIA, SUSPICIOUS FOR UROTHELIAL CARCINOMA. SECTIONS OF THE LEFT POSTERIOR AND RIGHT POSTERIOR MARGINS ARE BENIGN ()
PERMANENT DIAGNOSIS: SAME

5. SP: BLADDER MARGIN (FS): NEGATIVE FOR CARCINOMA ()
PERMANENT DIAGNOSIS: SAME

BCG > 90 day
to procurement.

Criteria	lw 2/6/14	Yes	No
Diagnosis Discrepancy			
Primary Tumor Site Discrepancy			
HIPAA Discrepancy			
Prior Malignancy History			
Dual/Synchronous Primary Malignancy			
Case is (circle):	QUALIFIED	DISQUALIFIED	
Reviewer Initials			